

PUBLIC HEALTH POLICY

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

Global Data · 2023 · IHME Global Burden of Disease

Hundreds of Millions Affected — Mental Illness Is a Universal Burden Spanning the Globe

Key lifetime prevalence estimates · Source: Our World in Data / IHME GBD 2025



1 in 3

women will experience
major depression
in their lifetime

Lifetime prevalence · IHME Global Burden of Disease 2025

Higher lifetime risk driven by hormonal, psychosocial, and socioeconomic vulnerability factors



1 in 5

men will experience
major depression
in their lifetime

Lifetime prevalence · IHME Global Burden of Disease 2025

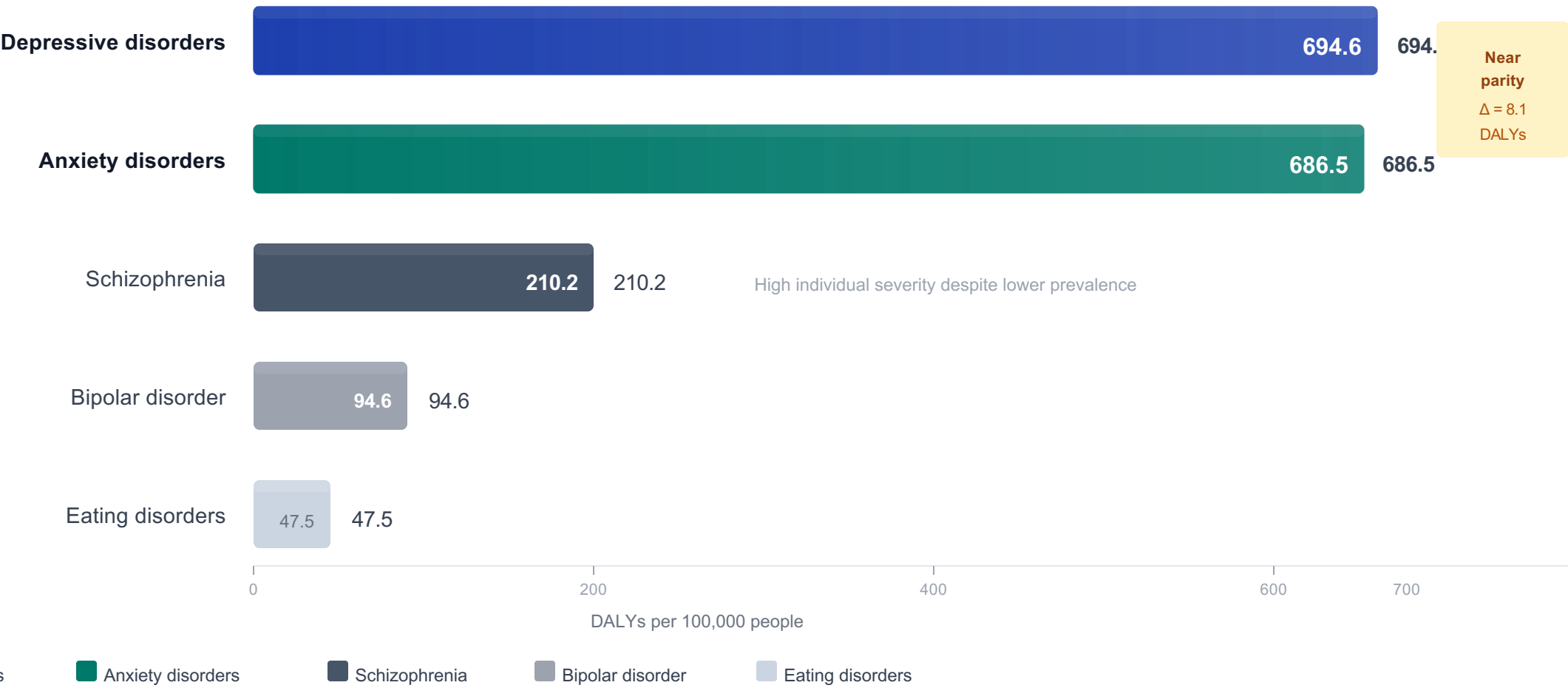
Risk is not limited to high-income settings — mental illness affects people across all countries and income levels

Why This Matters for Policy

Mental health conditions span from highly common (depression, anxiety) to less prevalent but profoundly severe (schizophrenia, bipolar disorder). Hundreds of millions suffer each year; many more across their lifetimes. Despite effective treatments existing, most people do not receive adequate care — making this a critical target for evidence-based public health investment.

Depression and Anxiety Dominate: ~695 and ~687 DALYs per 100K — Together ~80% of Mental Health Burden

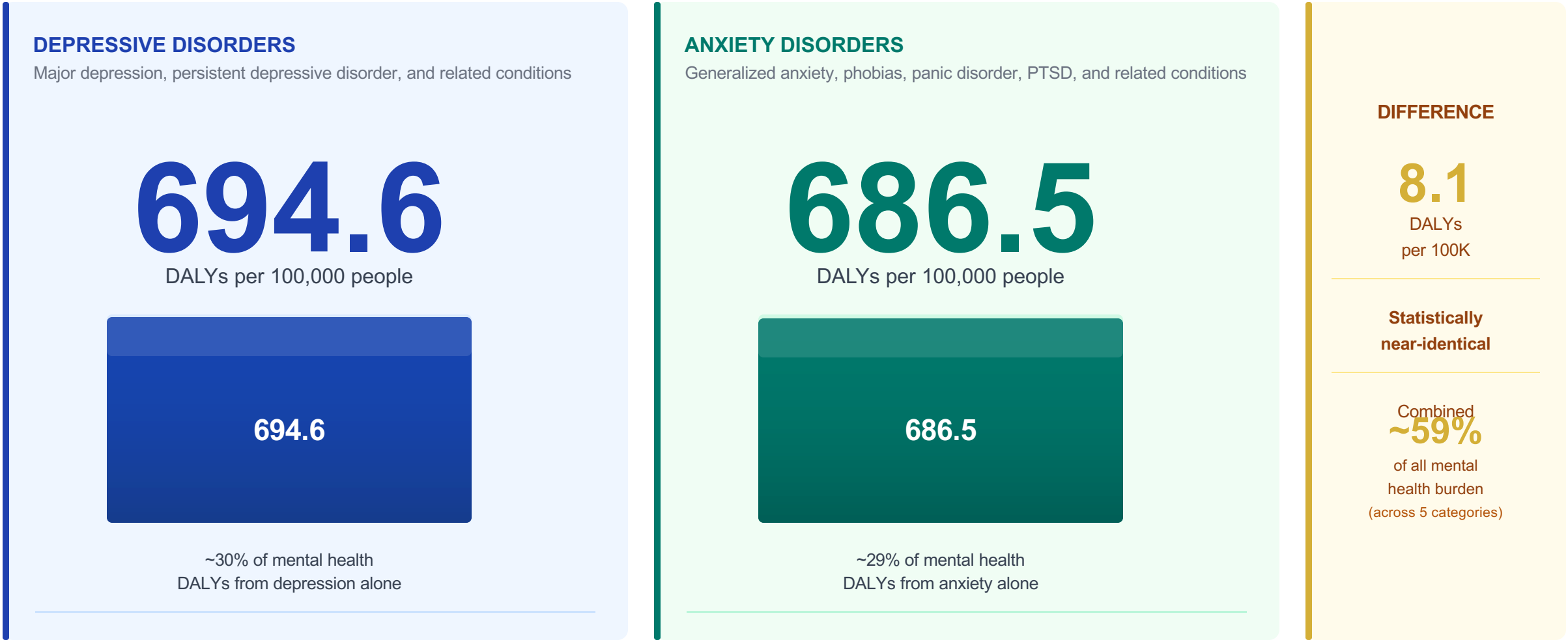
Disability-Adjusted Life Years (DALYs) per 100,000 people · World · 2023 · IHME GBD 2025



DALYs = Disability-Adjusted Life Years: combines years of life lost to premature death + years lived with disability. A higher DALY rate = greater population burden.

Depression and Anxiety Carry Nearly Equal Disease Burden — Together They Account for the Vast Majority

DALYs per 100,000 people · World · 2023 · Side-by-side comparison



Policy implication: The near-equal burden of depression and anxiety means both must be treated as co-equal policy priorities, not secondary to severe mental illness.

Schizophrenia and Bipolar: Lower Population Burden but Disproportionately High Individual Severity

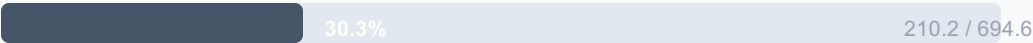
DALYs per 100,000 people · World · 2023 · In context against depression/anxiety

SCHIZOPHRENIA

210.2

DALYs per 100,000 people

vs. Depressive disorders (694.6):



▪ Profound disability

Positive symptoms (psychosis), negative symptoms, and cognitive impairment combine to create severe burden

▪ Early onset, lifelong trajectory

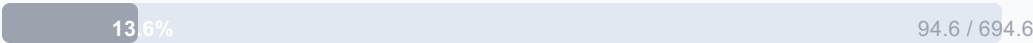
Typically onset in late teens/early twenties; requires long-term specialist care and community support

BIPOLAR DISORDER

94.6

DALYs per 100,000 people

vs. Depressive disorders (694.6):



▪ Episodic but functionally disabling

Cycles of mania/hypomania and depression impair occupational and social functioning even between episodes

▪ Often underdiagnosed

Frequently misdiagnosed as depression before bipolar features become apparent; delayed treatment is common

Disease burden in context (all 5 categories):



Policy implication:

While schizophrenia and bipolar carry a smaller share of population-level DALYs, their per-person severity demands specialized, sustained care systems — not just primary care scaling.

Treatment Is Available but Often Lacking or of Poor Quality — A Large Gap Persists Globally

Mental illnesses are **treatable**. Yet across all income settings, many people do not receive adequate care — or any care at all.

FOUR KEY BARRIERS TO TREATMENT



STIGMA

Many people are uncomfortable disclosing mental health symptoms to healthcare professionals or even to people they know.

Stigma suppresses help-seeking and distorts prevalence data — the true burden is likely higher than reported.



ACCESS BARRIERS

Treatment is often lacking, especially in low- and middle-income countries where mental health systems are severely underfunded.

Geographic, financial, and systemic barriers prevent millions from reaching trained mental health providers.



QUALITY GAPS

Even where treatment exists, it may be of poor quality — underdosed medications, inadequate psychotherapy, or untrained providers.

Receiving some care is not the same as receiving effective, evidence-based care.



UNDERREPORTING

Discomfort with disclosure makes it difficult to accurately estimate the true prevalence of mental illness — actual burden is likely underestimated.

Good data is essential: how, when, and why conditions occur, and how many people are truly affected.

How People Cope: From Medication to Faith to Family Conversations — Strategies Vary Widely

Global survey data reveals wide variation in how people deal with anxiety or depression · Source: Our World in Data / Global Survey Data

THREE DOMINANT COPING STRATEGIES IDENTIFIED IN GLOBAL SURVEYS



Key insight:

The diversity of coping strategies underscores that mental health responses are shaped by cultural, economic, and healthcare system factors. Policy interventions must account for these differences rather than applying a one-size-fits-all approach.

KEY TAKEAWAYS: PRIORITIZING DEPRESSION, ANXIETY, AND CLOSING THE TREATMENT GAP



Depression and anxiety carry the dominant share of global mental health burden

~695 and ~687 DALYs per 100,000 people respectively — near-identical in magnitude, together representing ~59% of the five-category burden. These conditions must be co-equal policy priorities at scale.



Schizophrenia and bipolar disorder carry disproportionately severe individual burden

Despite contributing 210.2 and 94.6 DALYs/100K — lower in population terms — their per-person severity demands specialized, sustained care systems. Scaling primary care alone is insufficient.



Effective treatments exist — but access and quality remain critically inadequate

Especially in low- and middle-income countries. Investment must address both the availability and evidence-base quality of mental health care delivery systems.



Stigma suppresses both help-seeking behavior and accurate epidemiological data

The true burden is likely higher than reported figures suggest. Anti-stigma campaigns and safe disclosure pathways are prerequisites for both care access and data quality.



Policy should scale evidence-based care for common disorders while maintaining specialized services

Task-shifted and culturally adapted interventions for depression and anxiety at primary care level, combined with sustained specialist capacity for severe conditions.

Data source:

Our World in Data — Dattani, Rodés-Guirao, Ritchie & Roser (2023) · IHME Global Burden of Disease 2025